

How Pokémon has shaped neuroscience

The Pokémon media franchise has influenced various aspects of science, from ecology and evolution to research integrity (see *Nature* **651**, 285–286; 2026). It has also provided a natural experiment in developmental and cognitive neuroscience, showing how shared childhood experience can affect brain organization.

Since Pokémon's release in 1996, pixelated creatures have bombarded the retinas and cortices of millions of children for thousands of hours as they have held their devices at arm's length. The gameplay loop thus sent a consistent visual stimuli in a natural environment across time. A study showed that adults who played the game extensively in childhood have distinct and spatially consistent functional representations in the visual cortex compared with people lacking such experience (J. Gomez *et al. Nature Hum. Behav.* **3**, 611–624; 2019).

Crucially, where this brain response emerged depended on how the images were viewed in childhood – how small and how centrally focused they were on the retina. This suggests that early, patterned visual input helps to determine how the brain organizes what we see. Responses to the 2019 study further highlight broader implications for developmental plasticity and visual specialization (D. Janini and T. Konkle *Nature Hum. Behav.* **3**, 552–553 (2019); G. Reyes *Nature Rev. Psychol.* **4**, 621; 2025).

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Rethink AI's role in survey research

An artificial-intelligence bot can now pass 99.8% of attention-check questions in surveys (see *Nature* **650**, 17; 2026). But this needn't be a threat: these checks were invented to catch careless human respondents. AI did not create the data-quality problem – it has simply made it harder to ignore.

As researchers who have studied survey methodology for five years, we know the issue of human contamination well. Even without AI tools, respondents often fill in only the minimum requirements, answer carelessly or adjust responses so that they seem socially desirable.

By contrast, AI responses offer transparency. A study that replicated 156 psychological experiments using large language models found that they were efficient for pilot testing and hypothesis validation (Z. Cui *et al. Nature Comput. Sci.* **5**, 627–634; 2025).

But AI tools are not human. When research aims to understand how people feel and experience their lives, no simulation can be a substitute. Human participants are therefore a scarce resource worth protecting.

Whereas AI systems can serve in pilot testing and tool refinement, humans can be reserved for studies that truly require them. The reduced need for frequent human participation might yield more careful, engaged responses. The goal is not to replace humans, but to deploy them more wisely.

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Health systems must recognize autism in older adults

Only a tiny proportion of autism research focuses on adults (see *Nature* **650**, 538–540; 2026). Psychiatrists are increasingly treating older individuals whose autism was not recognized in earlier life. A late diagnosis can provide clarity and relief, but also raises questions about future health risks and support.

Emerging data suggest higher rates of heart disease, Parkinson's-like symptoms, osteoporosis and dementia in autistic adults (B. N. Hand *et al. Autism* **24**, 755–764; 2020), although intellectual disability, long-term psychotropic use and diagnosis bias can blur links. Systematic investigation is needed. Without identification and tailored screening pathways, health systems cannot anticipate risk or adapt care.

Three priorities seem urgent. First, inclusion: autistic adults must not be excluded from ageing research, dementia studies or population-health data sets. Second, differentiation: clinicians require guidance to distinguish lifelong neurodevelopmental differences from neurodegenerative change. Third, co-production: older autistic adults should shape research agendas and service models to ensure meaningful support, not simply diagnosis.

As populations age globally, psychiatry, geriatric medicine and primary care must be prepared to take autism in older adults into account – through training, adapted assessment tools and neurodiversity-informed models of care.

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Marine conservation cities: a model for ocean governance

An article you published this month highlighted fast-rising sea levels (see K. Seeger and P. S. J. Minderhoud *Nature* <https://doi.org/qtzc>; 2026). These are one of many pressures oceans and coastal communities face – from pollution and overfishing to coastal development and climate change. Coastal cities might look to others driving ocean-sustainability innovations. Along Germany's Baltic coast, the councils of Kiel and Flensburg have adopted formal resolutions to become 'marine conservation cities' (in 2020 and 2022, respectively).

Although the specifics are still under development, this label will designate a city that establishes marine protection as a municipal responsibility and integrates it into political and urban decision-making. Cities will be able to pursue this protection in several domains – underwater, on the water and in the urban environment. Actions will include habitat restoration, pollution control, sustainable tourism and education.

I argue that this emerging model is noteworthy beyond regional symbolism. Coastal cities worldwide contribute to, and are affected by, marine degradation and climate change. Together with national and international frameworks, city-level commitments can accelerate local action, raise public awareness and promote ocean justice.

As coastal urbanization expands, city-led marine protection can bridge science, policy and civic engagement.

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